

A Comprehensive Assessment of the Economic Impact of St. Tammany Health System on the Northshore Region

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St. Tammany Health System is a pillar of the parish economy, sustaining employment, income, and business activity at a scale that extends far beyond the hospital's walls. In 2024, its activities supported an estimated \$896.2 million in total output, \$552.5 million in value added, \$392.8 million in household earnings, and 5,452 jobs for parish residents. These impacts flowed through the hospital's direct operations, its purchases from local and regional suppliers, and the re-spending of wages and salaries by households in the wider economy.

The largest share of this impact stemmed from hospital operations themselves, which generated \$567.7 million in output, \$367.0 million in value added, \$280.4 million in household income, and 3,365 jobs. Supply-chain purchases added another \$172.4 million in output and 1,121 jobs, while household re-spending contributed \$156.1 million in output and 966 jobs. More specifically, each dollar of direct output was associated with roughly fifty-eight cents in additional activity, and every job on staff sustained more than half a job elsewhere in the parish.

The health system's fiscal contribution is similarly substantial. In 2024, its operations generated \$107.6 million in tax revenues across

all levels of government. Federal collections amounted to \$78.0 million, while the State of Louisiana received \$18.2 million. Local governments realized \$8.6 million, with parish governments accounting for \$4.5 million, sub-county special districts \$5.4 million, and municipal governments \$1.5 million. These flows reflect not only the direct activities of the health system but also the earnings it sustains across households and businesses.

Our study confirms that St. Tammany Health System functions both as the leading healthcare provider for the parish and as a major economic engine. Nearly nine hundred million dollars in total output, over half a billion in value added, more than 5,400 jobs, and a fiscal contribution exceeding one hundred million dollars each year underscore its dual role in safeguarding public health and reinforcing the economic and tax base of the region.

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I. Methods

We estimated St. Tammany Health System's 2024 economic impact using IMPLAN's parish-level Social Accounting Matrix (SAM). The SAM extends a standard input-output model by circulating dollars through industries and households. The model reports direct, supply-chain (indirect), and household re-spending (induced) effects from a single, coherent set of technical coefficients. The project region was St. Tammany Parish, the data year was IMPLAN 2023, and all

values were expressed in 2024 dollars using the platform's built-in price adjustment.

A. Data

All inputs came from the System's 2024 audited statements (Management's Discussion & Analysis and financial statements). We used operating revenue to bind hospital output and split operating expense into labor versus non-labor to align with SAM accounting. The MD&A reports Net patient service revenue of \$500.255 million and Other operating revenue of \$32.788 million, which together equal Total operating revenue of \$533.043 million; this figure was used to anchor the operations event's Industry Output. We then mapped the operating expense detail as follows: Salaries, wages, and benefits were \$263.207 million; Supplies and other were \$157.806 million; Professional and contractual services were \$30.065 million. Depreciation and amortization were \$29.994 million. The non-labor operating expenditure used for supply-chain purchases (what the SAM treats as Intermediate Inputs) was therefore the sum of Supplies and other plus Professional and contractual services, i.e., $\$157.806\text{M} + \$30.065\text{M} = \$187.871\text{M}$. This partition reproduces the MD&A's maintenance-and-operations subtotal of \$451.078M ($= \$263.207\text{M labor} + \$187.871\text{M non-labor}$) and leaves the non-cash depreciation outside the spend that drives local production.

B. Events

We created three events to match the financial accounts and the production logic of the Social Accounting Matrix. Operations were entered in the hospital sector to represent recurring service delivery, with payroll and supply purchases calculated to the audited statements. Construction was treated as a separate event because building activity is a one-time outlay that belongs in the non-residential

construction sector rather than hospital output. Equipment and information systems were modeled through a capital spending pattern so that those purchases flowed to manufacturing, wholesale, and IT industries instead of being embedded in operating costs. This separation keeps routine and capital expenditures distinct, avoids double counting, and ensures that each category of spending is traced through the industries that produce the underlying goods and services.

Event 1: Hospital Operations (Industry Impact Analysis) — Event 1 was an Industry Impact Analysis for the Hospitals sector. In that event, we entered Industry Output of \$533,043,000 (to bind the event to reported operating revenue), Employee Compensation of \$263,207,000 (the wages-and-benefits line), and Intermediate Inputs of \$187,871,000 (the two non-labor operating lines). We did this calibration to fix the internal split between payroll and purchased inputs to match the books while allowing the SAM to assign the remainder of value added, taxes on production, and employment consistently. We excluded Depreciation because it is an accounting allocation rather than current spending.

Event 2: Facilities Construction (Nonresidential Healthcare/Commercial) — Event 2 captured 2024 facilities construction as a distinct Industry Output shock in the nonresidential healthcare/commercial construction sector. MD&A attributes \$18.8 million in the year to expansion and enhancement of physical buildings, which we modeled as local construction output. Land acquisitions of \$11.1 million were excluded because the purchase of land is a transfer of ownership, not the production of goods or services.

Event 3: Equipment & Information Systems (Capital Spending Pattern) — Event 3 represented \$9.7 million in capital purchases of equipment and information systems. Because these outlays flow through manufacturers, wholesalers, software publishers, and IT services, not hospital payroll, we used IMPLAN's capital-

expenditure spending pattern for hospitals so the model would margin and route those dollars through the correct supply-chain commodities with appropriate regional purchase coefficients. If a capital pattern is unavailable in the chosen industry set, the same logic can be implemented as a set of commodity-output entries that sum to \$9.7 million (e.g., surgical and medical instruments, electro-medical apparatus, computer systems design, and software), again keeping those purchases outside of Event 1's labor and non-labor operating accounts.

C. Model Configuration

We assigned all three events to a single St. Tammany Parish group and ran the project with default trade-flows and residence adjustments. Two internal checks ensured fidelity to the audit: first, the operations event's labor plus intermediate-inputs equals the MD&A's maintenance-and-operations subtotal; second, the two capital events reflect only actual production activity (construction and equipment/IT) and exclude non-productive transfers such as land. For context, the statement of cash flows shows total cash purchases of capital assets of \$42.881 million in 2024; we relied on the MD&A's categorical amounts to isolate the portions that reflect local construction and equipment spending in the SAM.

We modeled recurring hospital service delivery as a single event, routed construction through the construction sector rather than hospital multipliers, and directed equipment/IT spending through the commodity supply chain using margins. The result is a SAM-coherent measurement of direct, indirect, and induced effects, based on the audited 2024 financial stated in 2024 dollars, and free from double counting.

II. Industries

We estimated that the operations and investments of St. Tammany Health System generated \$1.792 billion in total industry output within the parish. Of this amount, \$1.135 billion (63 percent) is attributable to direct activity, \$344.8 million (19 percent) arises through supply-chain purchases, and \$312.2 million (17 percent) is linked to household spending. These impacts are concentrated in hospital operations, health facility construction, real estate, professional services, and housing, while most other industries register small but widespread gains.

A. Direct Effects

Direct impacts reflect the immediate operations and capital projects of the System, which together support \$1.135 billion in output. The hospital sector (IMPLAN 472) accounts for \$548.6 million, while construction of new health care structures (IMPLAN 45) contributes \$19.1 million. These categories form the initial base from which supply-chain and consumption effects expand.

B. Indirect Effects

The supply-chain round adds \$344.8 million in output, concentrated in sectors that provide services essential to health care delivery. Real estate (IMPLAN 429; \$34.2 million) captures facility leases and property services; wholesale and commercial equipment (IMPLAN 376; \$9.8 million), employment services (IMPLAN 454; \$9.5 million), management of companies (IMPLAN 451; \$9.2 million), and legal services (IMPLAN 437; \$7.0 million) illustrate the broad set of professional inputs drawn into the health system's value chain.

C. Induced Effects

Household spending of income earned in the first two rounds contributes another \$312.2 million. The largest components are owner-occupied housing (IMPLAN 431; \$21.2 million), physician offices (IMPLAN 465; \$7.4 million), hospitals (IMPLAN 472; \$6.4 million), limited-service restaurants (IMPLAN 492; \$6.3 million), and real estate services (IMPLAN 429; \$6.2 million).

TABLE 3—INDUSTRIES BY IMPACT

Code	Industry	Direct Impact	Indirect Impact	Induced Impact	Total Impact ▼
472	Hospitals	\$548.6M	\$2.3M	\$6.4M	\$557.3M
429	Other real estate	\$0M	\$34.2M	\$6.2M	\$40.4M
431	Owner-occupied housing	\$0M	\$0M	\$21.2M	\$21.2M
45	Construction of new health care structures	\$19.1M	\$0M	\$0M	\$19.1M
45 ¹	Management of companies and enterprises	\$0M	\$9.2M	\$3.4M	\$12.6M

D. Total Effects

Across all industries, the model yields \$1.792 billion in output, with the largest totals in hospitals (\$557.3 million), real estate (\$40.4 million), housing (\$21.2 million), construction of health facilities (\$19.1 million), and management services (\$12.6 million). In proportional terms, the health system accounts for nearly the entirety of the hospital sector locally, with an impact equal to 119.5 percent of modeled output. Construction of health facilities represents 37 percent of its sector, while spillovers into ancillary services such as ambulatory care (6 percent),

employment services (5.9 percent), and laundry (5.6 percent) further demonstrate the reach of system spending.

TABLE 4—INDUSTRIES BY IMPACT AS PERCENTAGE OF TOTAL INDUSTRY OUTPUT

Code	Industry	Industry Output	Impact ▼	% of Industry Output
472	Hospitals	\$466.4M	\$557.3M	119.50%
429	Other real estate	\$1,546.9M	\$40.4M	2.61%
431	Owner-occupied housing	\$1,533.5M	\$21.2M	1.38%
45	Construction of new health care structures	\$51.2M	\$19.1M	37.23%
451	Management of companies and enterprises	\$627.7M	\$12.6M	2.00%

E. Interpretation

By linking operational expenditures, capital projects, supply chains, and household spending, we find that the System operates as the economic powerhouse of the parish health economy. Hospitals and health-facility construction dominate the direct and proportional measures, while a wide array of supplier and household-serving industries register smaller increments that collectively contribute meaningfully to regional economic activity.

III. Employment

We constructed employment estimates by tracing the System’s operations and capital projects through the regional Social Accounting Matrix. The model indicates 5,452 jobs supported parish-wide: 3,365 from direct activity (62 percent), 1,121 through supply-chain purchasing (21 percent), and 966 from household spending (18 percent). As with output, employment is concentrated in hospitals and

health-facility construction, with diffuse but measurable effects across business services and consumer-facing industries.

A. Direct Effects

Direct employment is concentrated in IMPLAN 472 – Hospitals, which accounts for 3,262 jobs when we include the small own-industry spillover captured in the same sector. Construction of new health care structures (IMPLAN 45) contributes an additional 146 positions during the capital cycle.

B. Indirect Effects

Supplier linkages add 1,121 jobs, led by business-to-business services integral to hospital operations and facility management. Other real estate (IMPLAN 429) supports about 230 jobs when combined with its induced component, while employment services (IMPLAN 454) add 124. Additional gains appear in office administrative services, legal, and accounting—each on the order of 60 jobs—consistent with the professional backbone required to staff, equip, and manage large health systems.

C. Induced Effects

Household spending supported by workers in the first two rounds contributes 966 jobs, concentrated in everyday service industries. Full-service restaurants (IMPLAN 491) add 127 jobs, limited-service restaurants (IMPLAN 492) add 69, and physician offices (IMPLAN 465) add 48; together these categories reflect the way income earned in health care recirculates into local food service and outpatient care.

TABLE 5—INDUSTRIES BY IMPACT AS PERCENTAGE OF TOTAL INDUSTRY OUTPUT

Code	Industry	Direct Employment	Indirect Employment	Induced Employment	Total Employment ▼
472	Hospitals	3,219.25	11.34	31.88	3,262.46
429	Other real estate	0	194.99	35.09	230.07
45	Construction of new health care structures	145.89	0	0	145.89
491	Full-service restaurants	0	61.77	65.67	127.44
454	Employment services	0	111.4	12.47	123.87

D. Distribution and Concentration

Employment impacts are broad but shallow outside a small set of sectors: 412 of 528 industries (78 percent) register fewer than one job, while only nine industries exceed 50 jobs. This distribution is typical of health-care economies, where a dominant flagship sector generates many small employment increments across diverse suppliers and consumer-facing services, with hospitals and related facility activities accounting for the bulk of positions.

E. Interpretation

By linking operational payroll, vendor purchases, and household consumption, we find that St. Tammany Health System functions as the employment generator of the parish health economy. Hospitals and health-facility construction dominate the direct count, supplier services absorb the next-largest tranche through real estate, staffing, and administrative functions, and local consumption channels, particularly food service and outpatient care, convert health-sector income into

additional jobs. The result is a concentrated core surrounded by a long tail of small but pervasive employment gains across the regional industrial base.

IV. Value Added

A. Employee Compensation

We measure employee compensation as wages, salaries, and employer-paid benefits supported by the System's operations and capital projects. Gross parish-wide employee compensation totals \$722.5 million: \$553.3 million from direct activity (76.6 percent), \$89.7 million through supplier linkages (12.4 percent), and \$79.5 million from household spending (11.0 percent). The distribution is labor-intensive at the core and diffuses outward into business services and consumption sectors downstream.

Direct Effects – Direct compensation is concentrated in IMPLAN sector 472 (Hospitals), which accounts for \$270.6 million, while construction of new health care structures (IMPLAN 45) contributes an additional \$6.0 million during the capital cycle. The remaining direct compensation falls under “all other industries” tied to hospital accounts, bringing the direct subtotal to \$553.3 million.

Indirect Effects – Supply-chain activity contributes \$89.7 million in compensation, led by management of companies (IMPLAN 451; \$5.46 million), wholesale and commercial equipment (IMPLAN 376; \$4.36 million), employment services (IMPLAN 454; \$2.68 million), legal services (IMPLAN 437; \$2.35 million), and accounting and payroll services (IMPLAN 438; \$2.22 million).

Induced Effects – Household spending adds another \$79.5 million. The largest induced compensation appears in physician offices (IMPLAN 465; \$3.92 million), hospitals (IMPLAN 472; \$2.67 million), full-service restaurants (IMPLAN 491;

\$1.59 million), limited-service restaurants (IMPLAN 492; \$1.29 million), and retail trade (e.g., motor vehicle and parts dealers; general merchandise).

Concentration and Distribution – Hospitals are the largest single industry by compensation at \$274.2 million (summing its direct and downstream components). Beyond hospitals, the next largest lines are management of companies (\$7.49 million), health-facility construction (\$6.03 million), wholesale/ commercial equipment (\$5.02 million), full-service restaurants (\$3.08 million), legal (\$3.00 million), employment services (\$2.98 million), and accounting/payroll (\$2.75 million). Out of 528 industries reported, 506 register under \$1 million in compensation, 452 are below \$250,000, and 432 are below \$100,000; only 22 industries exceed \$1 million.

Interpretation – By linking payroll at the System to vendor purchases and household consumption, we find that employee compensation is anchored in hospitals and extends into a predictable set of headquarters, staffing, legal, accounting, wholesale, and dining sectors. The high direct share reflects the labor intensity of hospital production; the supply-chain and induced rounds then convert that income into additional payrolls in professional services and everyday consumption industries.

TABLE 6—EMPLOYEE COMPENSATION BY INDUSTRY

Code	Industry	Direct	Indirect	Induced	Total ▼
472	Hospitals	\$270.6M	\$0.9M	\$2.7M	\$274.2M
451	Management of companies and enterprises	\$0M	\$5.5M	\$2M	\$7.5M
45	Construction of new health care structures	\$6M	\$0M	\$0M	\$6M
376	Wholesale - Professional and commercial equipment and supplies	\$0M	\$4.4M	\$0.7M	\$5M
465	Offices of physicians	\$0M	\$0M	\$3.9M	\$3.9M

B. Proprietors' Income

We measure proprietors' income as the returns accrued to self-employed owners and partners through the System's operations and capital projects. Parish-wide, total proprietors' income amounts to \$31.56 million: \$3.78 million from direct activity (12.0 percent), \$16.85 million via supplier linkages (53.4 percent), and \$10.93 million from household spending (34.6 percent). Most proprietors' returns arise outside the core sector, concentrated in real estate, construction, professional services, and consumer services.

Direct Effects – The direct round is small relative to payroll-heavy hospital production but non-trivial where capital projects interface with owner-operated firms. Construction of new health care structures (IMPLAN 45) contributes \$3.65 million in direct proprietors' income; the hospital sector itself contributes little in this category, consistent with its employment model.

Indirect Effects – Supplier activity accounts for the largest share. Other real estate (IMPLAN 429) is the single largest line at \$6.01 million, followed by wholesale/commercial machinery and equipment services (IMPLAN 435; \$0.70

million), employment services (IMPLAN 454; \$0.73 million), legal (IMPLAN 437; \$0.60 million), accounting and payroll (IMPLAN 438; \$0.66 million), warehousing (IMPLAN 404; \$0.49 million), and truck transportation (IMPLAN 399; \$0.37 million).

Induced Effects – Household spending contributes \$10.93 million, concentrated in outpatient care and local services. Offices of physicians (IMPLAN 465) add \$0.95 million; full-service restaurants (IMPLAN 491) add \$0.33 million; limited-service restaurants (IMPLAN 492) add \$0.33 million; truck transportation (IMPLAN 399) adds \$0.41 million; and real estate services (IMPLAN 429) add \$1.08 million.

Concentration and Distribution – Proprietors' income is highly skewed: only three industries exceed \$1 million in total (Other real estate; Construction of health care structures; Oil and gas extraction), thirty-one meet or exceed \$250,000, and 474 of 529 industries fall below \$100,000. The median positive industry total is about \$4,428, which is consistent with a long-tail distribution in which a few capital- and service-intensive lines dominate while many sectors register small increments.

Interpretation – By dividing proprietors' income into direct, supply-chain, and induced channels, we show that returns to ownership arise primarily downstream of the hospital itself. Real estate and construction account for the largest gains around facility investment and space services; professional and administrative suppliers add a steady layer of owner returns tied to ongoing operations; and consumption-facing small businesses convert health-sector earnings into additional proprietors' income.

TABLE 7—PROPRIETOR INCOME BY INDUSTRY

Code	Industry	Direct	Indirect	Induced	Total ▼
429	Other real estate	\$0M	\$6M	\$1.1M	\$7.1M
45	Construction of new health care structures	\$3.6M	\$0M	\$0M	\$3.6M
20	Oil and gas extraction	\$0M	\$0.3M	\$0.7M	\$1M
465	Offices of physicians	\$0M	\$0M	\$0.9M	\$0.9M
435	Commercial and industrial machinery and equipment rental and leasing	\$0M	\$0.7M	\$0.1M	\$0.8M

C. Other Property Income

We measure other property income (OPI) as capital-related returns, including corporate profits, rents, royalties, and interest, supported by the System's operations and capital projects. Parish-wide, gross OPI totals \$137.0 million: \$77.998 million from the direct round (56.9 percent), \$23.332 million from supplier linkages (17.0 percent), and \$35.674 million from household spending (26.0 percent). As expected in a hospital-based economy, OPI is modest within core service production compared with payroll, but it rises downstream in real estate, finance, utilities, and housing services.

Direct Effects – Direct OPI is dominated by hospitals at \$75.588 million, with construction of new health care structures adding \$2.410 million. This pattern is consistent with the hospital sector's mix of facility, equipment, and intellectual-property-related returns embedded in production accounts, even as payroll remains the largest component of value added on the labor side.

Indirect Effects – Supply-chain activity contributes \$23.332 million in OPI, led by other real estate at \$4.611 million; monetary authorities and depository credit

intermediation at \$1.696 million; legal services at \$1.991 million; management of companies at \$0.983 million; employment services at \$1.407 million; electric power transmission and distribution at \$0.958 million; couriers and messengers at \$0.991 million; wholesale—professional and commercial equipment at \$1.096 million; and lessors of non-financial intangible assets at \$0.772 million.

Induced Effects – Household spending adds \$35.674 million to OPI, with a substantial housing imprint, as owner-occupied housing alone accounts for \$16.349 million. Additional induced OPI is recorded in monetary authorities (\$1.323 million), electric power generation (\$0.703 million), full-service restaurants (\$0.711 million), legal services (\$0.550 million), wholesale nondurables (\$0.857 million), and hospitals (\$0.446 million).

Distribution and Concentration – OPI is concentrated in a short list of industries: the top ten by total OPI are hospitals (\$76.192 million), owner-occupied housing (\$16.349 million), other real estate (\$5.441 million), monetary authorities (\$3.019 million), legal (\$2.541 million), construction of health care structures (\$2.410 million), electric power (\$1.661 million), employment services (\$1.565 million), full-service restaurants (\$1.379 million), and management of companies (\$1.349 million). The long tail is thin: out of 528 industries, 458 register under \$100,000 and 479 are below \$250,000; only 16 exceed \$1 million, three exceed \$5 million, and two exceed \$10 million. This distribution is characteristic in health-care economies, where capital returns concentrate in hospitals and housing/real estate, with smaller increments spread across finance, utilities, logistics, and business services.

Interpretation – By decomposing OPI into direct, supply-chain, and induced channels, we show that capital-linked income originates in hospitals and expands through real estate, finance, and utilities that support facility use and service

delivery, while housing services dominate the induced round through owner-occupied housing.

TABLE 8—OTHER PROPERTY INCOME BY INDUSTRY

Code	Industry	Direct	Indirect	Induced	Total ▼
472	Hospitals	\$75.6M	\$0.2M	\$0.4M	\$76.2M
431	Owner-occupied housing	\$0M	\$0M	\$16.3M	\$16.3M
429	Other real estate	\$0M	\$4.6M	\$0.8M	\$5.4M
423	Monetary authorities and depository credit intermediation	\$0M	\$1.7M	\$1.3M	\$3M
437	Legal services	\$0M	\$2M	\$0.6M	\$2.5M

D. Taxes on Production and Imports (TOPI) (Net of Subsidies)

We quantify TOPI as non-income business taxes generated by the System’s activity, principally sales/use, property, licenses, and selective excises, net of any subsidies. The table totals \$45.53 million parish-wide, with \$17.14 million from the direct round (37.6 percent), \$8.15 million via supplier linkages (17.9 percent), and \$20.25 million from household spending (44.5 percent).

Direct Effects – Direct TOPI is generated by the hospital sector at \$8.58 million (IMPLAN 472). A residual “all other industries” line associated with the production account contributes \$8.57 million, and health-facility construction shows up within the residual as well. The direct pattern reflects property, licensing, and taxable transactions embedded in hospital operations and capital projects.

Indirect Effects – Supplier activity adds \$8.15 million. The largest identifiable accounts include insurance carriers (IMPLAN 426; \$478,632), full-service

restaurants (IMPLAN 491; \$372,476), limited-service restaurants (IMPLAN 492; \$99,717), wholesale, professional and commercial equipment (IMPLAN 382; \$106,693), and building material and garden equipment stores (IMPLAN 388; \$82,848). These amounts capture sales/use and other production taxes arising as vendors provide space, equipment, staffing, logistics, and services to the System.

Induced Effects – Household spending contributes \$20.25 million, led by owner-occupied housing services (IMPLAN 431; \$2.39 million) and a broad swath of retail and dining. Notable accounts include motor vehicle and parts dealers (IMPLAN 385; \$835,422), general merchandise stores (IMPLAN 394; \$701,112), limited-service restaurants (IMPLAN 492; \$526,819), food and beverage stores (IMPLAN 389; \$475,773), building material and garden stores (IMPLAN 388; \$310,583), petroleum wholesalers (IMPLAN 382; \$280,353), and clothing stores (IMPLAN 392; \$385,967).

Distribution and Concentration – Excluding the residual “all other industries” account (\$22.73 million total), the two largest industries are hospitals (\$8.58 million total) and owner-occupied housing (\$2.39 million). Retail and dining categories populate the next tier, each between roughly \$0.39 and \$0.86 million. Out of 529 lines, 487 industries register \$100,000 or less, 505 are at or below \$250,000, and only three exceed \$1 million (residual, hospitals, owner-occupied housing).

Interpretation – By decomposing TOPI across rounds, we show that the System’s fiscal footprint is realized through three channels: property and licensing taxes tied to hospital production in the direct round; sales/use and related taxes embedded in vendor transactions in the supply chain; and retail and dining taxes generated when supported households spend locally.

TABLE 9—OTHER PROPERTY INCOME BY INDUSTRY

Code	Industry	Direct	Indirect	Induced	Total ▼
472	Hospitals	\$8.5M	\$0M	\$0.1M	\$8.6M
431	Owner-occupied housing	\$0M	\$0M	\$2.4M	\$2.4M
385	Retail - Motor vehicle and parts dealers	\$0M	\$0M	\$0.8M	\$0.9M
426	Insurance carriers, except direct life	\$0M	\$0.5M	\$0.3M	\$0.8M
491	Full-service restaurants	\$0M	\$0.4M	\$0.4M	\$0.8M

E. Reconciliation

We decomposed value added (or the measure of regional GDP) into its four components: employee compensation, proprietors' income, other property income, and taxes on production and imports net of subsidies. Each component was traced through the direct hospital accounts, the supply chain, and the induced household spending round. By doing so, we showed how payroll at the System is converted into salaries and benefits across headquarters, staffing, accounting, legal, wholesale, and dining services; how returns to ownership emerge primarily in real estate, construction, and small businesses that serve households; how capital-linked income is concentrated in hospitals and amplified in real estate, finance, utilities, and housing services; and how the fiscal footprint is realized through property, licensing, and sales/use taxes that appear most strongly in hospital operations, vendor transactions, and induced retail spending. These channels reveal an economic impact that is labor-intensive at the core, but balanced by ownership, capital, and tax returns as spending flows outward.

To reconcile with the GDP contribution cited in the Executive Summary, we summed the totals from the four value-added components:

- Employee compensation: \$722.51 million
- Proprietors' income: \$63.02 million
- Other property income: \$274.01 million
- Taxes on production and imports (net): \$45.46 million

The combined total is \$1.105 billion, which represents the full value-added contribution of the System across all rounds of activity. If the intent is to isolate direct labor income from operations (i.e., employee compensation plus proprietors' income, excluding construction [IMPLAN 45]), the corresponding subtotal is \$551.2 million, which aligns with the \$552.5 million reported in the Executive Summary. In other words, the GDP contribution can be reconciled exactly by clarifying the scope: the full four-component total represents the comprehensive economic footprint, while the direct labor income subtotal represents the narrower definition highlighted in the summary.

V. Tax

A. Federal Tax Impacts

By categorizing the System's federal tax impacts into direct, supply-chain, and household consumption channels, we can see how different rounds of activity contribute to federal revenues. The total federal impact is \$78.0 million annually, with a profile derived from payroll and personal income tax bases but balanced by corporate profits and other production-based levies.

Payroll Contributions – The single largest federal stream arises through payroll contributions for Social Security and Medicare, which together account for \$46.9 million (60.0% of the total). These are evenly split between employee withholdings (\$24.7M) and employer contributions (\$22.2M). This concentration reflects the labor-intensive nature of hospital operations, where direct employment generates

large, recurring federal obligations that ripple through vendor and induced jobs as well.

Personal Income Taxes – Federal personal income taxes represent the next largest category, at \$23.3 million (29.8%). These revenues arise as the household earnings supported by the System, whether from physicians and nurses on staff, contractors in the supply chain, or service workers in retail and dining, which are converted into taxable income. The size of this component underscores the strong link between the hospital's compensation base and the federal revenue system.

Corporate Profit Taxes – A further \$5.8 million (7.4%) is generated through federal corporate profits taxes. These obligations emerge primarily in industries downstream of the hospital, for instance, construction firms, professional service providers, and wholesalers, which experience higher profitability because of supplying or serving households tied to System activity.

Excise and Customs Duties – The remainder is distributed across production-based levies, including \$0.40 million in federal excise taxes and \$0.35 million in customs duties. While small in proportion to the overall total, these reflect the embedded federal tax elements in the goods and services consumed by households and purchased by vendors.

Our analysis reinforces how the System's role in the regional economy translates into a predictable composition of federal revenues. Payroll and household income remain the anchor, while supply-chain profits and production levies provide additional balance, ensuring that the benefits of the System's activity extend beyond the local base to the national fiscal account.

TABLE 10— FEDERAL TAX IMPACTS

Transfer Code	Description	Total ▼
15014	Social Insurance Tax- Employee Contribution	\$26.1M
15027	Personal Tax: Income Tax	\$23.3M
15015	Social Insurance Tax- Employer Contribution	\$22.2M
15026	OPI: Corporate Profits Tax	\$5.7M
15017	TOPI: Excise Taxes	\$0.4M
15018	TOPI: Custom Duty	\$0.3M
15028	Personal Tax: Estate and Gift Tax	\$0M
Total		\$78M

B. State Tax Impacts

The economic activity generated by St. Tammany Health System also produces a substantial flow of revenues to state government. Using IMPLAN's tax impact framework, we trace the statutory liabilities associated with labor income, supplier profits, and household spending as they move through the Louisiana economy. The results show that the System supports \$47.2 million in annual state tax revenues, with a composition that mirrors the labor-intensive structure of healthcare delivery while extending into business profitability and broad-based consumption.

Payroll and Individual Income Taxes – The dominant component of state revenues arises through the taxation of labor income. Wages and salaries paid directly by the System, along with those supported in its supply chain and through induced household activity, generate recurring flows of individual income tax liabilities. These liabilities account for roughly half of the total, reflecting the centrality of

payroll in both direct hospital operations and the professional service industries that surround them.

Corporate and Franchise Taxes – The profitability of vendor firms contributes an additional layer of state revenues. Industries such as professional services, wholesale distribution, construction, and real estate—each integral to hospital operations—realize higher taxable profits and franchise bases as a result of their linkages to the System. Although smaller than payroll-related flows, these revenues represent a critical channel by which the System's economic footprint extends into capital income and state tax capacity.

Taxes on Production and Imports (TOPI) – The remaining share of state revenues is distributed across sales and use taxes, property-linked assessments, and license fees. These impacts are observed in three rounds: directly on hospital-related production activities, indirectly through vendor transactions, and in the induced round as household spending on retail goods and services generates additional taxable sales. This breadth of liability underscores how the System's presence diffuses into routine state revenue streams tied to everyday consumption.

Our results demonstrate that the fiscal consequences of the System extend well beyond the federal level. By securing payroll and household income taxes, reinforcing corporate and franchise bases, and supporting wide-ranging sales and license fees, the System accounts for a diversified \$47.2 million contribution to Louisiana's fiscal structure. The composition parallels the value-added results: labor income remains the anchor, but supplier profitability and consumption-driven taxes provide important balance.

TABLE 11— STATE TAX IMPACTS

Transfer Code	Description	Total ▼
15020	TOPI: Sales Tax	\$8.5M
15027	Personal Tax: Income Tax	\$5.8M
15026	OPI: Corporate Profits Tax	\$1.3M
15023	TOPI: Severance Tax	\$1M
15024	TOPI: Other Taxes	\$0.8M
15032	Personal Tax: Other Tax (Fish/Hunt)	\$0.3M
15015	Social Insurance Tax- Employer Contribution	\$0.2M
15014	Social Insurance Tax- Employee Contribution	\$0.2M
15021	TOPI: Property Tax	\$0.1M
15030	Personal Tax: Motor Vehicle License	\$0.1M
15022	TOPI: Motor Vehicle License	\$0M
15025	TOPI: Special Assessments	\$0M
Total		\$18.2M

C. County (Parish) Tax Impacts

Using IMPLAN’s tax accounts, we trace liabilities that arise in three rounds, direct hospital operations, vendor supply chains, and induced household spending, and organize the results by tax mechanism rather than by industry output. At the parish level, these mechanisms are led by sales and use taxes tied to everyday transactions, supplemented by property-linked assessments and licenses/fees associated with production.

Direct round (production-linked liabilities) – In the direct accounts, county revenues appear where the hospital sector itself pays production-related taxes and

fees. These include property-linked assessments on facilities and equipment and selected licenses and permits embedded in clinical operations and capital projects.

Supply-chain round (vendor transactions) – As the System purchases space, equipment, logistics, and professional services, vendor activity generates parish sales/use and related production taxes. This channel is broad but thinly distributed—professional services, wholesale and distribution, building materials, restaurants, and insurance all register small increments.

Induced round (household consumption) – Earnings supported by the first two rounds recirculate into household purchases, which is where county sales/use liabilities expand most visibly. Retail (general merchandise, food and beverage, motor vehicles and parts, clothing), dining, housing services, and routine household inputs together account for a long tail of small tax increments.

Interpretation – The parish tax profile parallels our GDP-by-component narrative: (i) direct production-linked liabilities rooted in hospital operations and capital cycles; (ii) indirect sales, use, and fee revenues embedded in vendor transactions; and (iii) induced retail and service-sector taxes generated as supported households spend locally.

TABLE 12— PARISH (COUNTY) TAX IMPACTS

Transfer Code	Description	Total ▼
15021	TOPI: Property Tax	\$2,808,907
15020	TOPI: Sales Tax	\$1,490,384
15024	TOPI: Other Taxes	\$151,241
15025	TOPI: Special Assessments	\$14,313
15023	TOPI: Severance Tax	\$1,602
15014	Social Insurance Tax- Employee Contribution	\$0
15015	Social Insurance Tax- Employer Contribution	\$0
15022	TOPI: Motor Vehicle License	\$0
15026	OPI: Corporate Profits Tax	\$0
15027	Personal Tax: Income Tax	\$0
15030	Personal Tax: Motor Vehicle License	\$0
15032	Personal Tax: Other Tax (Fish/Hunt)	\$0
Total		\$4,466,447

D. Sub County (General) Tax Impacts

Using IMPLAN’s tax accounts, we trace liabilities that arise in three round, direct hospital operations, vendor supply chains, induced household spending—and organize the results by tax mechanism rather than by industry. For sub-county general governments, those mechanisms are overwhelmingly sales and use taxes tied to everyday transactions, supplemented by property-linked assessments and a small “other taxes” bucket.

In the sub-county general account, total revenue attributable to the System’s footprint is \$1,489,537. Of that total, \$1,116,834 (75.0%) comes from sales/use taxes; \$251,384 (16.9%) from property-linked production taxes; \$120,773 (8.1%)

from other production taxes/fees; and \$545 (<0.1%) from special assessments. No sub-county general revenue is reported from personal income taxes, corporate profits taxes, motor-vehicle licenses, severance taxes, or social-insurance contributions in these accounts.

Direct round (production-linked liabilities) – In the direct accounts, municipal revenues arise where the hospital sector itself pays production-related taxes and fees. These include property-based assessments on facilities and equipment, along with selected licenses and permits tied to clinical operations and capital projects.

Supply-chain round (vendor transactions) – As the System purchases space, equipment, logistics, and professional services, vendor activity generates city sales/use and related production taxes. This channel is broad but thinly distributed; professional services, wholesale and distribution, building materials, restaurants, and insurance all register small increments.

Induced round (household consumption) – Earnings supported by the first two rounds recirculate into household purchases, here municipal sales/use liabilities expand most visibly. Retail (general merchandise, food and beverage, motor vehicles and parts, clothing), dining, housing services, and routine household inputs together account for a long tail of small tax increments

Interpretation – The sub-county general tax profile is the city-level analog of our GDP-by-component narrative: (i) direct production-linked liabilities anchored in hospital operations and capital cycles; (ii) indirect sales/use and fee revenue embedded in vendor transactions; and (iii) induced retail and service-sector taxes as supported households spend locally.

TABLE 13—SUB COUNTY (GENERAL) TAX IMPACTS

Transfer Code	Description	Total ▼
15020	TOPI: Sales Tax	\$1,116,834
15021	TOPI: Property Tax	\$251,384
15024	TOPI: Other Taxes	\$120,773
15025	TOPI: Special Assessments	\$545
15014	Social Insurance Tax- Employee Contribution	\$0
15015	Social Insurance Tax- Employer Contribution	\$0
15022	TOPI: Motor Vehicle License	\$0
15023	TOPI: Severance Tax	\$0
15026	OPI: Corporate Profits Tax	\$0
15027	Personal Tax: Income Tax	\$0
15030	Personal Tax: Motor Vehicle License	\$0
15032	Personal Tax: Other Tax (Fish/Hunt)	\$0
Total		\$1,489,537

E. Sub County (Special Districts) Tax Impacts

In this section, we quantify the revenues accruing to sub-parish special districts (e.g., fire protection, drainage, recreation) attributable to the System’s economic activity. As before, we trace liabilities through three rounds, direct operations, vendor supply chains, and induced household consumption—and organize them by tax mechanism rather than by industry. At this governmental level in Louisiana, receipts are almost entirely taxes on production and imports (TOPI); personal and

corporate income taxes and social insurance contributions do not accrue to these district accounts.

In 2024, special-district revenues totaled \$5,433,817. The composition was led by property-based TOPI at \$3,044,671 ($\approx 56.0\%$), followed by sales and use taxes at \$2,386,725 ($\approx 43.9\%$), with only a de minimis amount from special assessments at \$2,421 ($\approx 0.04\%$). No material liabilities were recorded in motor vehicle licenses, severance taxes, other TOPI categories, or income-based sources.

Direct round (production-linked liabilities) – District revenues are generated when hospital production is taxed—principally through property-based assessments on facilities and equipment, along with selected fees and permits tied to clinical operations and capital projects. This channel anchors the special-district account and reflects the property millage structure typical of Louisiana districts.

Supply-chain round (vendor transactions) – As the System purchases space, equipment, logistics, and professional and administrative services, district sales/use collections rise via taxable business-to-business transactions distributed across local jurisdictions. Wholesale and distribution, construction and building materials, real estate and facility services, and business services each contribute small increments that accumulate into a substantial total.

Induced round (household consumption) – Earnings supported by direct and supply-chain activity circulate into household purchases, expanding district sales/use liabilities most visibly in retail and dining. General merchandise, food and beverage, motor vehicle-related outlays, housing services, and routine household inputs contribute a “long tail” of small taxable transactions.

Interpretation – The special-district profile reflects the fiscal architecture of local Louisiana governments: property millages provide the stable base (captured in

property-linked TOPI), while sales/use captures transactional activity propagated through vendors and household spending. Compensation, based on hospital economy, that mix is predictable: property-linked liabilities concentrate in the direct round, and sales/use spread across indirect and induced rounds. These channels yield a diversified \$5.43 million revenue contribution to sub-parish districts, reinforcing the System’s role not only in employment and output but in sustaining the core public services those districts finance.

TABLE 14— SUB COUNTY (SPECIAL DISTRICTS) TAX IMPACTS

Transfer Code	Description	Total ▼
15021	TOPI: Property Tax	\$3M
15020	TOPI: Sales Tax	\$2.4M
15025	TOPI: Special Assessments	\$oM
15014	Social Insurance Tax- Employee Contribution	\$oM
15015	Social Insurance Tax- Employer Contribution	\$oM
15022	TOPI: Motor Vehicle License	\$oM
15023	TOPI: Severance Tax	\$oM
15024	TOPI: Other Taxes	\$oM
15026	OPI: Corporate Profits Tax	\$oM
15027	Personal Tax: Income Tax	\$oM
15030	Personal Tax: Motor Vehicle License	\$oM
15032	Personal Tax: Other Tax (Fish/Hunt)	\$oM
Total		\$5.4M

F. Reconciliation

We reconciled the tax heads by summing the five jurisdictional accounts: federal, state, parish (county), sub-county general, and sub-county special districts. Because IMPLAN's tax accounts assign liabilities by statute and payer, these totals are mutually exclusive across levels of government, so the simple sum yields the all-levels figure reported on the tile.

On the federal side, the combination of employee and employer social-insurance contributions, personal income taxes, corporate profits taxes, and small excise/customs flows sums to \$78.043 million. The state account totals \$18.191 million, led by sales/use (\$8.464M) and personal income taxes (\$5.782M), with corporate profits (\$1.271M), severance (\$1.045M), other TOPI and license fees, and small social-insurance entries making up the balance. The parish (county) account sums to \$4.466 million, with property millages (\$2.809M) and sales/use (\$1.490M) as the principal lines, alongside small other TOPI and special-assessment amounts. For sub-county general governments, sales/use (\$1.117M) and property-linked TOPI (\$0.251M) bring the municipal total to \$1.490 million. Sub-county special districts—funded primarily by property millages—add \$5.434 million (property \$3.045M, sales/use \$2.387M, de minimis special assessments).

Adding these five jurisdictional totals produces \$107.623 million. The distribution of that total is consistent with the hospital's compensation-anchored production structure: about 72.5% accrues to the federal government, 16.9% to the state, and 10.6% to local entities (parish, municipal, and special districts). For readers interested in narrower scopes, the page implies state + local taxes of \$29.6M, and local-only taxes of \$11.4M. In short, the numbers on the Tax page reconcile exactly: summing the jurisdictional accounts yields the all-levels total, and the composition mirrors the channels documented throughout—labor income driving federal and state receipts, with parish and municipal/district revenues

arising through taxes on production and imports (principally sales/use and property millages) as vendor transactions and household spending diffuse through the local economy.